

1. Administrative & Study Identification

Full Study Title: A Clinical Study of Sacituzumab Tirumotecan (MK-2870) in Combination With Pembrolizumab (MK-3475) as First-line Maintenance Treatment of Cervical Cancer (MK-2870-036/TroFuse-036/GOG-3123/ENGOT-cx22) **Short Title/Acronym:** TroFuse-036 / MK-2870-036 **Protocol Number:** MK-2870-036 **NCT Number:** NCT07216703 **Sponsor/Company Name:** Merck **Investigational Product:** Sacituzumab Tirumotecan (MK-2870 / sac-TMT) and Pembrolizumab (MK-3475) **Phase of Development:** PHASE3 **Trial Status:** NOT_YET_RECRUITING (Not yet recruiting participants) **Medical Monitor:** Merck Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate if participants who receive sacituzumab tirumotecan (sac-TMT) and pembrolizumab (with or without bevacizumab) live longer overall or without their cancer getting worse compared to those receiving standard maintenance treatment. Also, to assess the safety and tolerability of the sac-TMT + pembrolizumab + bevacizumab combination. **Secondary Objectives:** To evaluate the Progression-free Survival 2 (PFS2), Objective Response Rate (ORR), Duration of Response (DOR), and other patient-reported quality of life metrics.

2.2 Background & Rationale To address the need for more effective treatments for metastatic cervical cancer, a major global health concern where advanced-stage options are limited. The study utilizes an antibody-drug conjugate (sac-TMT) which acts as targeted therapy to deliver a powerful anti-cancer agent directly to cells. This trial aims to establish whether using sac-TMT in combination with the immunotherapy pembrolizumab (+/- bevacizumab) as a first-line maintenance therapy can prolong survival while maintaining an acceptable safety profile.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Pembrolizumab maintenance) **Blinding:** Open-label (with single outcomes assessor masking) **Randomization:** Randomized, Parallel Assignment

3.2 Planned Enrollment & Duration Target **\$N\$:** 1,023 participants **Number of Sites:** Approximately 61 global locations **Estimated**

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Has a histologically confirmed diagnosis of squamous cell carcinoma, adenosquamous carcinoma, or adenocarcinoma of the cervix. 2. Has persistent, recurrent, or newly diagnosed metastatic cervical cancer that is not amenable to curative treatment (surgery and/or radiation). 3. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1, and tumor Programmed Cell Death Ligand 1 (PD-L1) expression of Combined Positive Score (CPS) ≥ 1 . 4. If infected with human immunodeficiency virus (HIV), has well-controlled HIV on antiretroviral therapy. 5. If positive for hepatitis B surface antigen, has received hepatitis B virus (HBV) antiviral therapy and has undetectable HBV viral load. If has a history of hepatitis C virus (HCV) infection, has undetectable HCV viral load.

4.2 Exclusion Criteria 1. Has a history of (noninfectious) pneumonitis/interstitial lung disease that required steroids or has current pneumonitis/interstitial lung disease. 2. Has HIV infection with a history of Kaposi's sarcoma and/or Multicentric Castleman's Disease, or active inflammatory bowel disease requiring immunosuppressive medication or previous history of inflammatory bowel disease (e.g., Crohn's disease, ulcerative colitis, or chronic diarrhea). 3. Has not adequately recovered from major surgery or has ongoing surgical complications. 4. Has a history of documented severe dry eye syndrome, severe Meibomian gland disease and/or blepharitis, or severe corneal disease that prevents/delays corneal healing. 5. Has uncontrolled, significant cardiovascular disease or cerebrovascular disease. 6. Has received prior systemic anticancer therapy other than what is specified in this protocol. 7. Has a diagnosis of immunodeficiency. 8. Has a known additional malignancy that is progressing or has required active treatment within the past 3 years. 9. Has known active central nervous system (CNS) metastases and/or carcinomatous meningitis. 10. Has an active autoimmune disease that has required systemic treatment in the past 2 years (replacement therapy allowed). 11. Is currently receiving a strong inducer/inhibitor of cytochrome P450 3A4 that cannot be discontinued. 12. Has a history of stem cell/solid organ transplant.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * *Induction:* Pembrolizumab 200 mg Q3W, paclitaxel 175 mg/m² Q3W, and cisplatin (Trade Name: Platinol, ATC Code: L01XA01) 50 mg/m² Q3W (or carboplatin AUC 5 Q3W) +/- bevacizumab 15 mg/kg Q3W. *

Maintenance: Sac-TMT 4 mg/kg Q2W and pembrolizumab 400 mg Q6W +/- bevacizumab 15 mg/kg Q3W. **Route of Administration:** Intravenous (IV) Infusion **Duration of Treatment:** Induction for up to 6 cycles (approx. 4 months). Maintenance for up to 14 cycles (approx. 19 months).

5.2 Concomitant & Prohibited Therapy Permitted: Bevacizumab is allowed throughout induction and maintenance at the investigator's discretion. Standard oncology supportive care medications. **Prohibited:** Strong inducers/inhibitors of CYP3A4 that cannot be discontinued; live-attenuated vaccines; concurrent investigational agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging, PD-L1 Testing
Baseline	Day 0	Allocation (Part 1 Safety Run-in) or Randomization (Part 2 Maintenance), First Dose
Interim	Q2W / Q3W / Q6W	Safety Labs, IV Infusions (cycle-dependent), Tumor Imaging (e.g., Q9W), AE assessment
End of Study	Up to 19 Months (Maintenance)	End of Treatment Visit, Final Vital Signs, Exit Interview, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * **Part 1 Safety Run-in:** 1. Number of Participants Who Experience One or More Adverse Events (AEs). 2. Number of Participants Who Discontinue Study Treatment Due to an AE. * **Part 2 Maintenance Treatment:** Progression-free Survival (PFS) per Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST 1.1) as Assessed by Blinded Independent Central Review (BICR). Defined as the time from randomization to the first documented progressive disease (PD) or death due to any cause.

7.2 Secondary & Exploratory Endpoints * **Part 2 Maintenance Treatment:** Progression-free Survival 2 (PFS2) as Assessed by the Investigator. * **Part 2 Maintenance Treatment:** Number of Participants Who Experience One or More AEs. * **Part 2 Maintenance Treatment:** Number of Participants Who Discontinue Study Treatment Due to an AE. * Objective Response Rate (ORR) and Duration of Response (DOR).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via physical examinations, vital signs, and laboratory assessments prior to each infusion cycle. **Serious Adverse Events (SAEs):** Reported

continuously from informed consent through 90 days after the last dose of study treatment. **Data Monitoring Committee (DMC):** An external, independent Data Monitoring Committee will oversee unblinded safety and efficacy data across the study.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy endpoints (PFS). Safety Set for all adverse events. **Sample Size Justification:** \$N \approx 1,023\$ participants ensures adequate statistical power to detect a significant survival benefit of adding sac-TMT to standard maintenance therapy. **Handling of Missing Data:** Participants without documented progression or death will be censored at the date of their last evaluable tumor assessment or date last known alive.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring to ensure protocol adherence, safety reporting, and data integrity. **Audit Trail:** Electronic Case Report Form (eCRF) utilizing compliant audit trails for all clinical and radiographic data.

11. Study Identifiers & Governance

Posting ID: 975996568883864995 **Primary ID:** MK-2870-036 **Registry Number:** NCT07216703 **Sponsor/Company:** Merck **Study Title:** TroFuse-036 **Official Regulatory Title:** A Phase 3, Randomized, Open-label, Multicenter Study to Evaluate the Efficacy and Safety of Sacituzumab Tirumotecan (MK-2870) in Combination With Pembrolizumab With or Without Bevacizumab Compared With Standard of Care as Firstline Maintenance Treatment for Participants With Persistent, Recurrent, or Newly Diagnosed Metastatic Cervical Cancer With PD-L1 CPS Greater Than or Equal to 1 (TroFuse-036/ GOG-3123/ENGOT-cx22)

12. Clinical Framework (Cervical Cancer Specific)

Primary Condition (Disease Area): Cervical Cancer **Diagnosis Threshold:** Histologically confirmed squamous cell carcinoma, adenosquamous carcinoma, or adenocarcinoma of the cervix, not amenable to curative treatment. **Medical Methodology:** 1st-Line Platinum-based Induction Chemotherapy + Immunotherapy followed by Maintenance therapy. **Optimization Target:** Maximize progression-free survival and overall survival using Antibody-Drug Conjugate (ADC) targeted delivery.

13. Study Procedures & Methodology

Management Pathway: 2-Part Study (Part 1: Safety run-in; Part 2: Standard induction followed by randomized maintenance). **Education Protocol (HCP):** Investigator training on managing potential ADC toxicities and immunotherapy-related AEs (irAEs). **Education Protocol (Patient):** Informed consent detailing standard-of-care induction vs. investigational maintenance options. **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for all progression-based endpoints to prevent bias.

14. Observation & Follow-Up Schedule

Total Duration: Up to 19 months active maintenance treatment; survival follow-up until study conclusion. **Onsite Visit Cadence:** Every 2 to 3 weeks during active treatment phases for drug administration. **Remote Monitoring Frequency:** Follow-up contacts for survival status post-treatment. **Checklist Review Interval:** Regular radiographic tumor assessments according to protocol schedules.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				